

From talking the talk to walking the walk

*Cultivating meaningful engagement
in a rural academic health system*

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Disclosures

No real or potential conflicts of interest.

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- The Promise Partnership Coproduction Learning Health System
- Connect Share Care virtual peer facilitated network
- The Beryl Institute
- The Patient Revolution

Overview

- Rosie Bartel's challenge
- Coproduction (barriers & facilitators)
- Structural engagement
- Engagement in improvement
- Engagement in teaching, learning, and scholarship
- Lessons learned and opportunities ahead

The challenge

*“I hear a lot of talk from
a lot of people...
... I want to see you
walk the walk.”*



PATIENT EXPERIENCE JOURNAL 2026;13(1):420-428

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SPECIAL SEGMENT

**From Talking the Talk to Walking the Walk:
Reflection on Dartmouth Health’s Care Experience Week**

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ABSTRACT

This article explores the participation of expert members from the Beryl Institute’s Global Patient and Family Advisory Board (GPFAB) during Dartmouth Health’s Care Experience Week, under the theme “From Talking the Talk to Walking the Walk.” This week aligns with the national Patient Experience Week. The Patient Family Advisor (PFA) presenters and authors of this article have shared their lived experience locally, nationally, and internationally through conferences, webinars, podcasts, videos, blogs, white papers, journal articles, books, and in many other ways for and as PFA members of a multitude of healthcare organizations co-create change that improves outcomes for all human beings. During this event, they emphasized the applications of co-design and coproduction in healthcare, not as theoretical ideals but as embedded operational practices. Drawing on powerful storytelling, structured sessions, and hands-on collaboration between leadership, staff, and patients, families, and care partners, the week advanced a model of human-centered care rooted in action. This article reflects on the insights gained, both affirming and challenging, and it offers practical pathways for embedding patient voice and lived experience at every level of the health system from the boardroom to the valet stand. In doing so, it underscores how the power of purposeful storytelling, inclusive patient and family advisory councils, and systemic co-design and coproduction can lead to real transformation in the patient and human experience.

Keywords:

Co-design, Coproduction, Lived experience, Patient experience, Human experience

1. Introduction: A week of living the values

Care Experience Week, which aligns with the national Patient Experience Week, in the Dartmouth Health system was not a conference. It served as a dynamic platform for collaboration and transformation in patient and human care. For those of us from the Beryl Institute’s Global Patient and Family Advisory Board (GPFAB), it was a chance to see whether an institution could genuinely walk the walk. Centered around the

theme “From Talking the Talk to Walking the Walk,” this week-long event highlighted the critical importance of authentic patient engagement through storytelling, using coproduction, and co-design. This theme set a high bar and the experiences they witnessed gave both cause for celebration and opportunities for growth. For members of the GPFAB, it was not only an opportunity to reflect on what is being done in the realm of patient partnership but also a call to action for the healthcare system

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Rosie Bartel, MA
(picture used with permission)

Bartel R, Kimpson C, Lord T, Lyons V, Serge T, Oliver BJ (2026). From Talking the Talk to Walking the Walk: Reflection on Dartmouth Health’s Care Experience Week. Patient Experience Journal; 13(1):420-428. doi: 10.35680/2372-0247.2095.

Coproduction

- “Service-dominant” logic
- Barriers
- Facilitators



OPEN ACCESS

Coproduction of healthcare service

Maren Batalden,¹ Paul Batalden,² Peter Margolis,³ Michael Seid,³ Gail Armstrong,⁴ Lisa Oipari-Arrigan,³ Hans Hartung⁵

ABSTRACT

Efforts to ensure effective participation of patients in healthcare are called by many names—patient centredness, patient engagement, patient experience. Improvement initiatives in this domain often resemble the efforts of manufacturers to engage consumers in designing and marketing products. Services, however, are fundamentally different than products; unlike goods, services are always ‘coproduced’. Failure to recognise this unique character of a service and its implications may limit our success in partnering with patients to improve health care. We trace a partial history of the coproduction concept, present a model of healthcare service coproduction and explore its application as a design principle in three healthcare service delivery innovations. We use the principle to examine the roles, relationships and aims of this interdependent work. We explore the principle’s implications and challenges for health professional development, for service delivery system design and for understanding and measuring benefit in healthcare services.

“The physician,” wrote Hippocrates, “must not only be prepared to do what is right [himself,] but also make the patient...cooperate.”¹ For centuries, traditional medical culture has recognised that some sort of patient partnership is essential. Recently, this partnership has received increasing attention. The Institute of Medicine named patient-centred care one of the six fundamental aims of the US healthcare system.² Empirical evidence suggests that informed, activated patients may be effective in facilitating good health outcomes at lower cost.³ Payers seek healthcare consumer evaluation of patient experience in their endeavour to measure and pay for value.⁴ The US Center for Medicare Services identifies patient and family engagement as a pillar in its efforts to improve healthcare.⁵

Contemporary dialogue about patient-centred care, however, seems

compromised by an implicit paradigm, which suggests that healthcare service is a product manufactured by healthcare systems for use by healthcare consumers. This product paradigm may confound efforts to put patients and professionals in right relationship. Healthcare service is better conceived as a service. Services, unlike manufactured goods, are always coproduced by service professionals and service users.

Even in the most traditional model of medical practice—patient comes to clinician for help, clinician listens to and examines the patient, clinician formulates a plan and instructs the patient, patient follows (or does not follow) suggestions—health outcomes (good and bad) are coproduced. Good outcomes, most recognise, are more likely if the patient can and does seek and receive help in a timely way, if the clinician and patient communicate effectively, develop a shared understanding of the problem and generate a mutually acceptable evaluation and management plan. The degree to which patients and professionals each hold agency for these coproduced outcomes varies widely, but the observation that health outcomes are a consequence of the dispositions, capacities and behaviours of both parties seems self-evident. Deceptively obvious, the concept has profound implications for improving healthcare quality, safety and value.

THE THEORY OF COPRODUCED SERVICES IN ECONOMICS, POLITICAL SCIENCE AND BUSINESS

Victor Fuchs noted in 1968 that the new service economy (eg, retail, banking, education, healthcare) was distinct from the old industrial economy (manufacturing, agriculture); services entail a different relationship between producer and consumer. Measuring productivity in the service economy, he noted, was

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CrossMark

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Walking the walk = meaningful engagement

- Structural engagement
- i3
- Teaching, learning, and scholarship

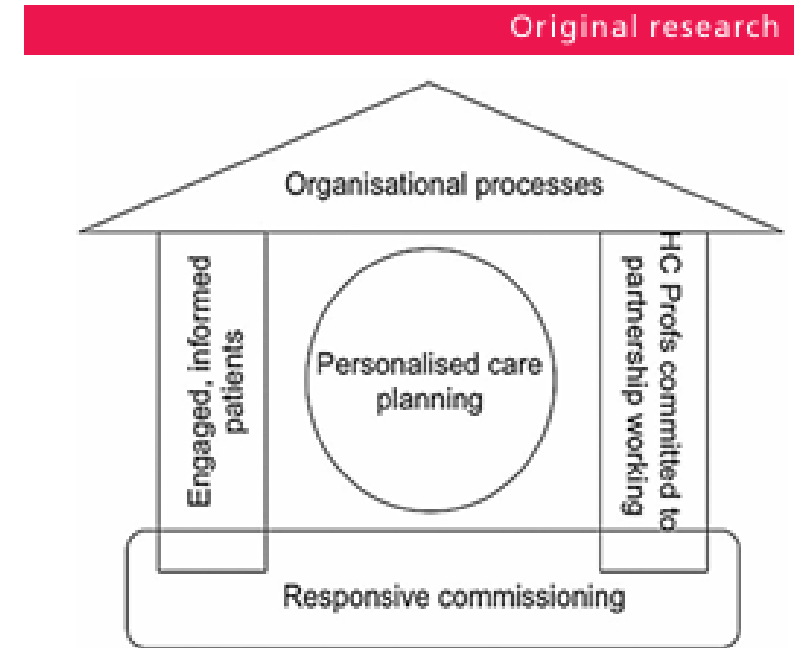


Figure 1 House of Care. Reproduced with permission of The King's Fund. Source: Coulter A, Roberts S, Dixon A (2013). Delivering better services for people with long-term conditions: building the house of care. London: The King's Fund. Available at: <http://www.kingsfund.org.uk/publications/delivering-better-services-people-long-term-conditions>

<https://qualitysafety.bmj.com/content/qhc/25/7/509.full.pdf>

Structural engagement

**Strategic
Plan**

**Committees
& Boards**

**Informing &
Evaluating
Care**

Academic

**Learning
Health
System**

**PFACs
&
CABs**

**Innovative
Volunteer
Initiatives**

Learning Health System



Together, we bring the full power of our collective expertise to provide the best possible care to our patients, our people, and our communities.

Dartmouth Health's strategic plan promises to draw on the expertise of clinical teams, researchers, educators, administrators, patients, families, and communities in an approach that sets DH apart from other health systems. To deliver on this promise, DH has partnered with The Dartmouth Institute for Health Policy and Clinical Practice to implement a Learning Health System based on principles of coproduction.

<https://sites.dartmouth.edu/promisepartnership/>

“PFAC-in-a-Box”

- Co-designed
- Science- and experience- informed
- Standardized and customizable
- Accessible via *ConnectShareCare*

<https://dh.connectsharecare.org/>

The screenshot shows a post on the ConnectShareCare platform. At the top, the 'ConnectShareCare' logo is on the left and the 'Dartmouth Health' logo is on the right. The post is titled 'PFAC Flashbuild Codesigned Best Practice Toolkit (2026)' and is attributed to 'Dr. Brant Oliver @boliver, Jun 22 3:27pm'. Below the title, a short description states: 'Developed via the falshbuild codesign process to bring the best of our external expertise and external international expertise together to create a best practices approach for Dartmouth Health'. A 'COMMENT' button is visible. A green navigation bar contains links for 'Home', 'Getting Started', and 'Support Groups', along with search, notification, and user icons. Below the navigation bar, a suggestion box says 'Interested in more discussions like this? Go to the PFAC Flashbuild group.' and a dropdown menu is set to 'Oldest to Newest'. The post content begins with 'Dr. Brant Oliver | @boliver | Jun 22 3:28pm' followed by the text: 'Current draft final materials (June 2026). Word format files are available on the CE Committee Shared drive folder (unable to post Word format to CSC). Shared files'. A list of shared files follows, each with a green link and a PDF icon: 'DH Goal Setting and Strategic Planning Final (DH-Goal-Setting-and-Strategic-Planning-Final-.pdf)', 'DH Charter Final (DH-Charter-Final.pdf)', 'DH Onboarding Appendix A (DH-Onboarding-Appendix-A.pdf)', 'DH Onboarding Appendix B (DH-Onboarding-Appendix-B.pdf)', 'DH Onboarding Appendix C (DH-Onboarding-Appendix-C.pdf)', 'DH Onboarding Appendix D (DH-Onboarding-Appendix-D.pdf)', 'DH Onboarding Appendix E (DH-Onboarding-Appendix-E.pdf)', 'DH Onboarding Final May 21 (DH-Onboarding-Final-May-21.pdf)', 'Expression of Interest-DH Clean (Expression-of-Interest-DH-Clean.pdf)', and 'PFAC Interview Matrix Clean May 18 (PFAC-Interview-Matrix-Clean-May-18.pdf)'. At the bottom, there are icons for 'Like', 'Helpful', and 'Hug', and a 'REPLY' button.

Patient & Family Voices

Enhancing PX with Real-Time Tech-Assisted Rounding: A Volunteer Initiative

- Joel King, Presenter
- Ruth Storey, Presenter
- Bridget Mudge, Presenter
- Marianne Barthel, Author
- Angela Price, Author
- Rhea Sachdeva, Author
- Brant Oliver, Author



Patient & Family Voices

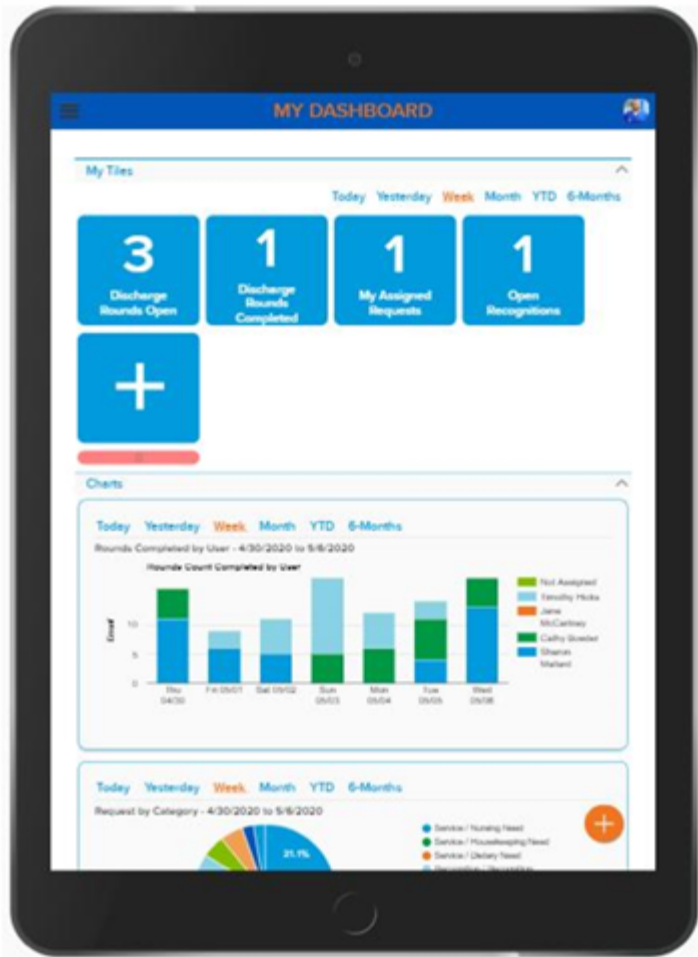
- Learn
- Personal Development
- Connecting
- Give Back
- Give Purpose
- For Meaning
- Improve the Care
- Listen
- Interacting
- Relationships with Peers
- Engaging
- Fun
- Community Engagement: Meet New People
- Staying Connected with Work I Love

Voices Volunteers...Why?



Voices Volunteers: Ruth, Sue, Karen, Joel, Bridget, and Bill. Not Present: Ada, Kaira, Rhea, and Sarah

Improvement Initiative



Joel King
Team Lead



Ruth Storey
Program Manager



Bridget Mudge
PFV Volunteer



Angela Price
Clinical Partner



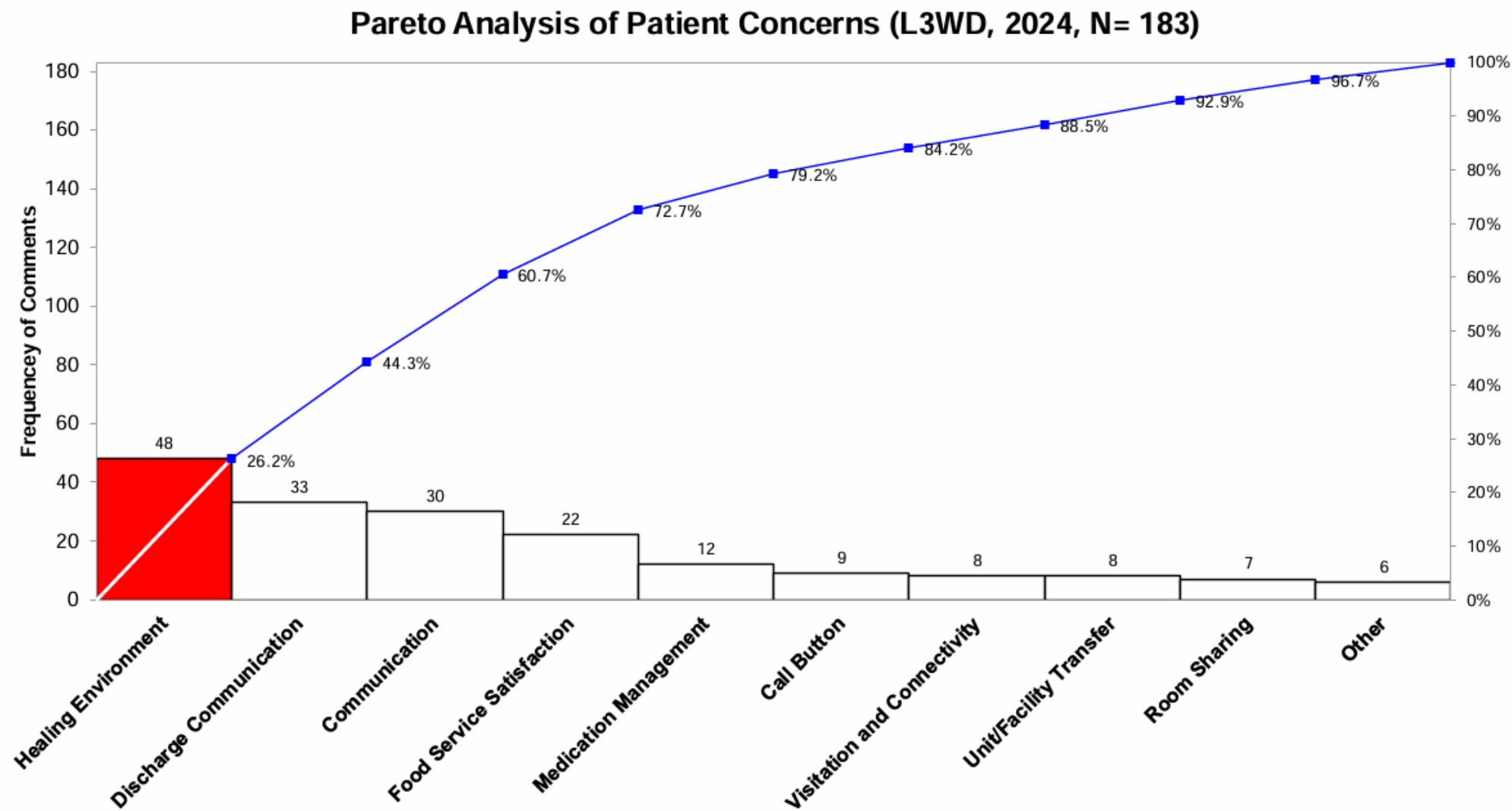
Marianne Barthel
Project Sponsor



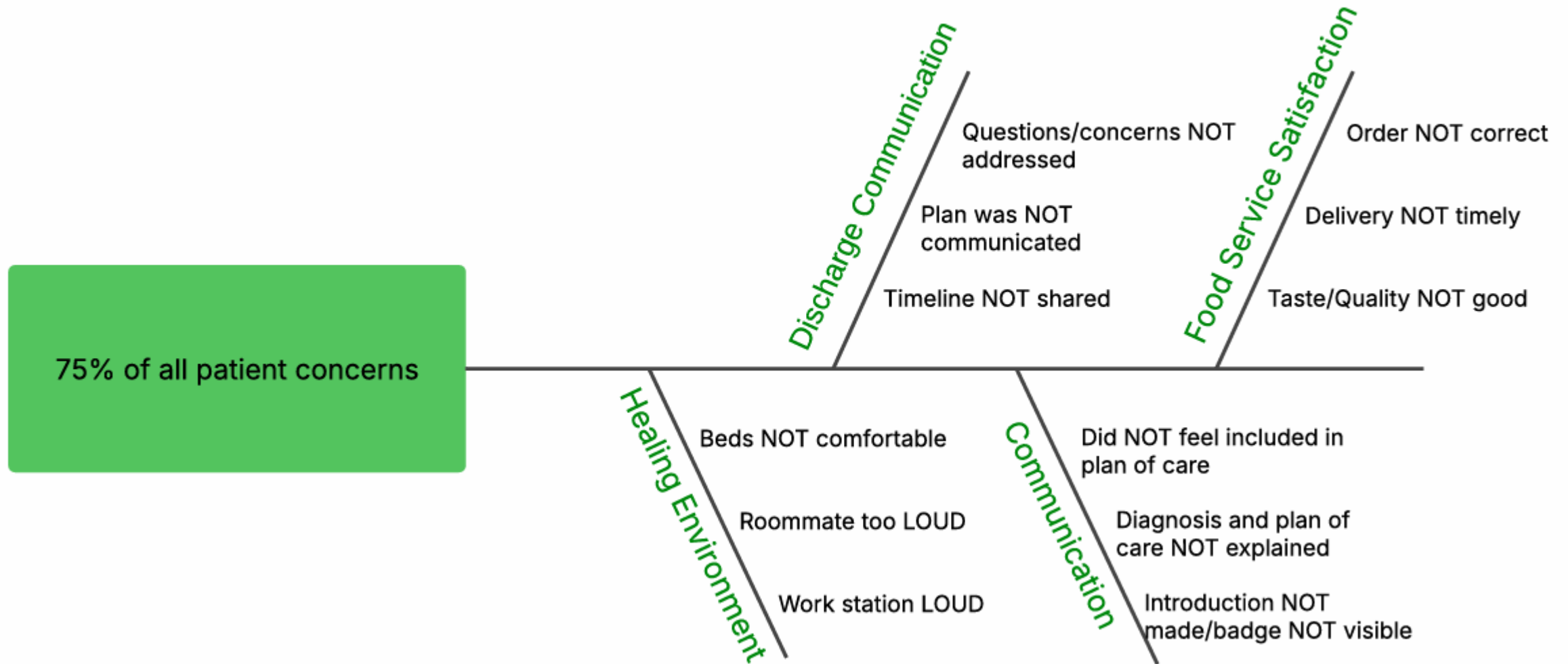
Brant Oliver,
LSSBB Coach

- Real-time rounding
- Technology assisted
- Feed-forward data for immediate action
- Feedback data for evaluation and improvement

Patient & Family Voices



Patient & Family Voices



Patient & Family Voices



Stock photo, not actual patient



Patient & Family Voices

Clinical Leadership Perspective



Angela Price, Unit Nurse Manager

“I find the PFV program very helpful. **The comments help myself and the staff on L3WD hear the stories/concerns directly from the patients.** This program allows me to fix any of the concerns that may be brought up **immediately** and also highlight the staff that the patients have acknowledged with the PFV staff. I would recommend this program to all units moving forward. It’s another set of eyes on our patients and it **helps address concerns in real time.**”

Groups

Follow the groups that interest you. Choose from the groups below or search by keywords. [See the m](#)

Search groups by keywords or complete phrases

Search

1

2



CHIRP i3 Sciences
Community

Following



CMC Well Being Council

Following



CTSA LHS CoP

Following

Implementation Collaborative

Our first implementation collaborative in safety at the Value Institute which will focus on safety and service behaviors across the health quality, safety, and experience outcomes.

L.Hubble@hitchcock.org
kennedy@dartmouth.edu

Translational Research Collaborative

Research Collaborative is a community of practice and works to mitigate barriers, network, support, scale, and adoption of evidence-based practices among all within our community. This group works in the clinical and academic realms to accelerate efforts in our Learning Health System including but not limited to, System Quality Improvement Leaders, SYNERGY investigators.

HOME ABOUT

Scientists Collaborative

This collaborative uses the following structure to support

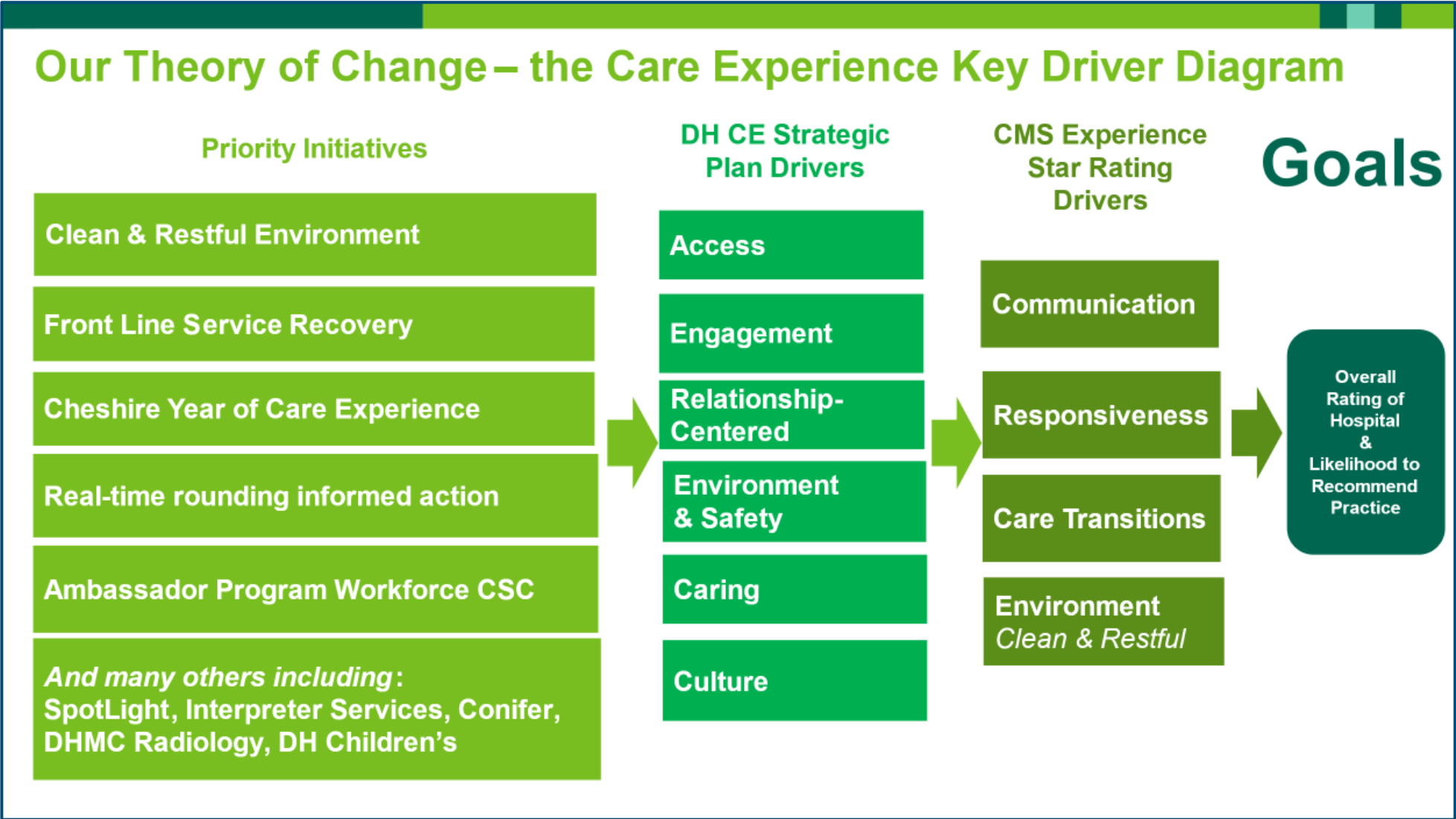
Learning Seminars open to the community focused on topics, followed by a private session for our scientists to presenters.

Community of practice working sessions providing informatics, and technical support to our LHS E-STAR to optimize their research, troubleshoot difficulties in and implementation, and build knowledge and skills access.

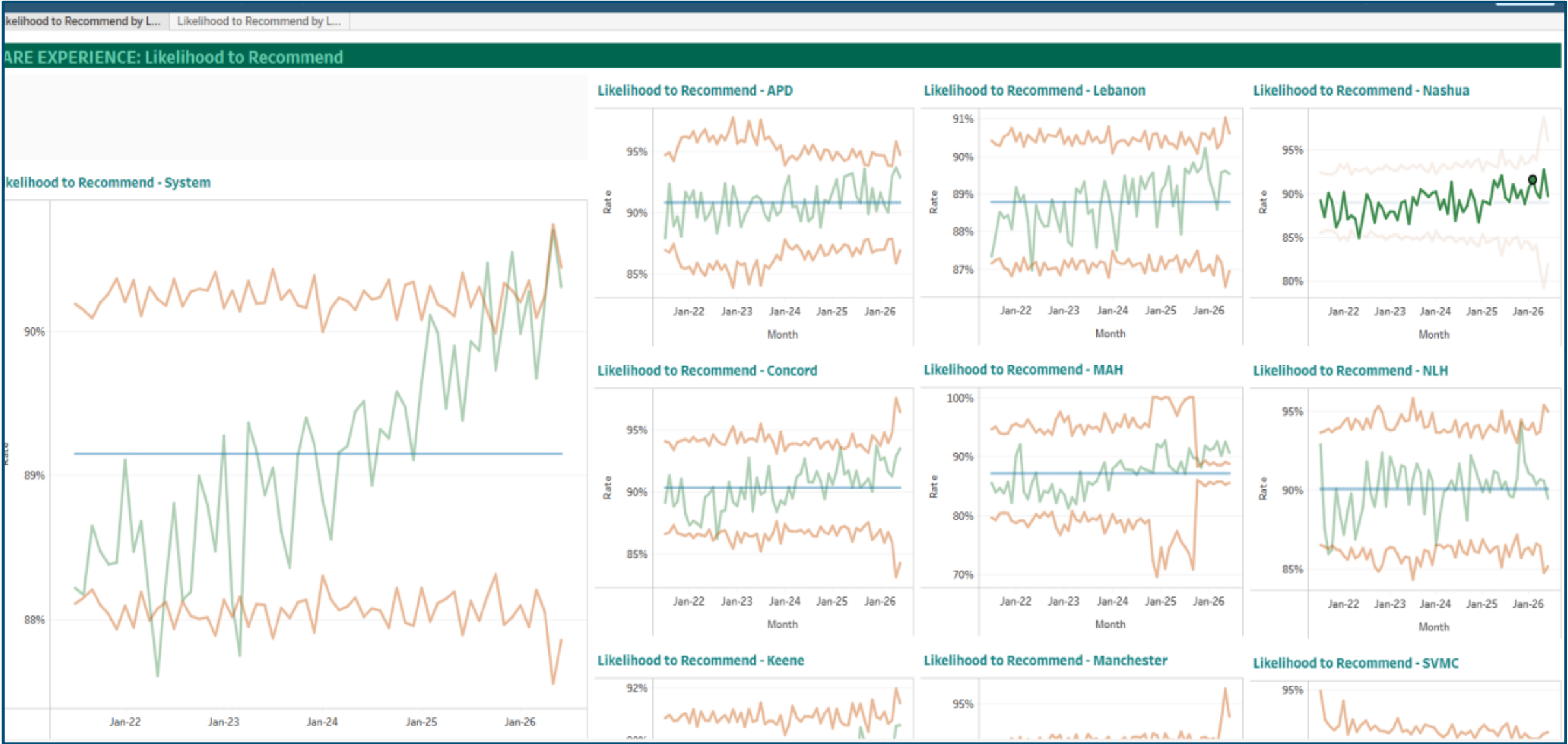
Learning Sessions that allow participants to share challenges, and lessons learned and identify new areas of discovery across Dartmouth's LHS.

- i3= “improvement, implementation, and innovation”
- Communities of Practice (Wenger)

i3 – Promise Partnership Care Experience Collaborative



i3 – Promise Partnership Care Experience Collaborative



i3: “Mini-Revolutions”

Care Experience Week Keynote Address

Apr 1, 2026

Keynote: Victor M. Montori, MD CAREFUL AND KIND CARE IN PRACTICE



About Programs Resources



**“We must transform
healthcare from an
industrial activity into a
deeply human one.”**

Victor Montori, MD



<https://sites.dartmouth.edu/promisepartnership/finding-your-place-key-takeaways-from-the-promise-partnership-coproduction-learning-health-system-fall-gathering/>

<https://www.patientrevolution.org/>

i3: Making the Business Case

- Quality
- Safety
- Experience
- Trust
- Reputation
- Financial



PATIENT EXPERIENCE JOURNAL 2024;11(1):14-20

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COMMENTARY

Investing in the Bottom Line: The Value Case for Improving Human Experience in Healthcare

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^a The Beryl Institute / Patient Experience Journal

^b Advocate Health

^c Vanderbilt University Medical Center

^d BJC HealthCare

^e Essentia Health

^f ECU Health

^g Dartmouth Health Care Experience & Geisel School of Medicine at Dartmouth

ABSTRACT

Investment in human experience is imperative for healthcare organizations. It is a strategic focus that can lead to great benefits. Those that overlook experience, seeing it as “simply” about satisfaction or survey data, do so at great cost to their organization, team members, and most importantly those we serve – patients. A commitment to human experience – integrating the patient, workforce, and community experience – is essential for all healthcare organizations in realizing the goals they strive for and the impact they aspire to achieve. It leads to high-quality outcomes for those they care for. It creates a positive environment for those who show up to serve each day. It fosters trust from the communities they serve. It is a central driver for financial viability and operational sustainability. It is a driver of quality outcomes, safety, and workforce engagement. It fosters consumer loyalty leading to stronger payor relationships, increased physician referrals, and greater patient, family, and consumer choice. Ultimately, a commitment to experience is the path to realizing a viable and thriving future for healthcare organizations. The concept of value in healthcare as initially proposed focused on healthcare outcomes per dollar spent. The reality is healthcare is a complex system driven by not only what is spent on patient outcomes, but also by how healthcare organizations engage with patients as people, care for their workforce, and address the needs of the communities they serve. A value case is not one solely about clinical outcomes but about the comprehensive healthcare experience through which a much broader set of outcomes is achieved. This paper proposes a model for the value of investment in human experience as a practical bottom-line issue for healthcare leaders.

Keywords: Human experience, Patient experience, Value case, Value, Return on investment, Workforce engagement, Patient engagement, Community engagement, Operational excellence

1. A commitment to human experience

Investment in human experience is imperative for healthcare organizations. It is a strategic focus that can lead to great benefits. Those that overlook experience, seeing it as “simply” about satisfaction or survey data, do so at great cost to their organization, team

members, and most importantly those we serve – patients.

A commitment to human experience – integrating the patient, workforce, and community experience¹ – is essential for all healthcare organizations in realizing the goals they strive for and the impact they

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- Teaching & certifying professionals (CPXP)
- Teaching with patient-partners (co-faculty)
- Designing and conducting research with patient-partner investigators
- Publishing with patient-partners: Partnership with the Beryl Institute and the PXJ for a two-part special section on improving human experience in healthcare.

1. Oliver BJ, Wolf J (May, 2026). Improving Human Experience in Health Care: Leveraging “Humanistic Learning Health Systems”; to Accelerate Change at Scale. Patient Experience Journal; 13(1):416-419. doi: 10.35680/2372-0247.2112.
2. Bartel R, Kimpson C, Lord T, Lyons V, Serge T, Oliver BJ (2026). From Talking the Talk to Walking the Walk: Reflection on Dartmouth Health’s Care Experience Week. Patient Experience Journal; 13(1):420-428. doi: 10.35680/2372-0247.2095.
3. Perry J, Hahn M, Kmiec A, Oliver BJ (2026). “We’ll be Right With You”; Improving Patient Experience With Complaint Management within a Learning Health System Collaborative. Patient Experience Journal; 13(1):429-439. doi: 10.35680/2372-0247.2113.
4. Dillehay R, Verkhovsky S, Bohm A, Friedman H, Welshman D, Morong A, Karthikeyan V, Dampier J, Racine A, Oliver BJ (2026). Documentation and Dashboard Driven Quality and Safety Improvement in Interpreter Services for Limited English Proficient Patients. Patient Experience Journal; 13(1):440-446. doi: 10.35680/2372-0247.2114.
5. Rizzo E, Adams V, Perry J, Szalay N, Verkhovsky S, Holthoff M, Kennedy A, Oliver BJ (2026). A Hybrid Coaching Model in a Multi-Center Networked Care Experience Learning Collaborative. Patient Experience Journal; 13(1):447-454. doi: 10.35680/2372-0247.2115.
6. Vallee J, Stewart J, Verkhovsky S, Giudice S, Aug J, Higgins A, Perry J, Kennedy A, Oliver BJ (2026). Enhancing Provider Engagement through Peer-to-Peer Coaching: The Provider Ambassador Program. Patient Experience Journal. 2026; 13(1):455-460. doi: 10.35680/2372-0247.2116.



INTRODUCTION TO SPECIAL SEGMENT

Improving Human Experience in Health Care: Leveraging “Humanistic Learning Health Systems” to Accelerate Change at Scale

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^b President & CEO, The Beryl Institute / Founding Editor, Patient Experience Journal

Human Experience (HX) closely aligns with the National Academy of Medicine (NAM) quality domain of person-centeredness² and improving HX should be a priority in healthcare quality. Unfortunately, after decades of work following the pivotal reports on healthcare quality and safety in the United States,^{3,4} substantive population-level improvements in the Quintuple Aim,^{5,6} outcomes (e.g. mortality, safety and life expectancy), cost and burden of care, patient experience, workforce experience, and health equity, have not yet been achieved at scale. This failure has not been due to a lack of rigorous technical methods^{7,15} but arguably to an overemphasis on the clinical, mechanistic and data informatics aspects of improvement work without equal emphasis on the human experience of the people engaged in healthcare.^{16,17} Montori and the Patient Revolution refer to this phenomenon as “industrialized healthcare.”¹

To improve experience at scale, The Beryl Institute Human Experience 2030 report¹⁸ calls for frame shifts from transactional (mechanistic) to relational (humanistic), aspiration to action, and siloed local work to networked scaled initiatives. One way to achieve this is to adapt powerful Learning Health System (LHS) approaches to become “humanistic engines” that can empower and accelerate experience improvement initiatives at scale. AHRQ defines a LHS as “a health system in which internal data and experience are systematically integrated with external evidence, and that knowledge is put into practice. As a

result, patients get higher quality, safer, more efficient care, and health care organizations become better places to work in and engage in care” (Fig. 1).¹⁹

LHS approaches can empower and accelerate Learning Collaboratives,²⁰ Communities of Practice,²¹ Coproduction Learning Health Systems,²² Learning Health Networks,^{23,26} and Storytelling.²⁷ The technical rigor of the “LHS as a machine” can be merged with the humanistic heart and soul of the “LHS as a complex adaptive organism.”²⁸ The result is a “humanistic LHS” which better serves and empowers the people within it, creates psychological safety, builds capability, and accelerates the improvement of HX at scale.

Included in a special segment in this issue of the *Patient Experience Journal*, are a series of reports on improving HX in healthcare supported by a humanistic LHS— the Dartmouth Promise Partnership Coproduction Learning Health System²⁹ in New Hampshire, USA. Promise is a strategic partnership between a medical school and a multi-center rural academic health system. It integrates translational science education and research, data informatics, and scaled and networked humanistic applied change initiatives. One of these initiatives is the *Care Experience Collaborative*, a multi-center improvement initiative across the health system focused on improving HX. Supported by Promise and guided by the care experience strategic plan for the health system, local

Lessons and opportunities

- Integrated approaches are cross-reinforcing and better sustaining.
- Technology-enhanced approaches can break through barriers but must be meaningfully connected to people.
- It must be part of the strategic plan.
- Build culture and communities developmentally.
- Opportunities to strengthen resilience and trust and enhance capability in context of uncertainty and austerity.
- A focus on people and culture rather than products and projects is critical.

Oliver BJ (2024). Improving Human Experience in Health Care: Now More Than Ever, We Must Focus on People. The Permanente Journal; 16;28(3):195-199. doi: 10.7812/TPP/24.125. Epub 2024 Sep 13. PubMed PMID: 39267443; PubMed Central PMCID: PMC11404654.

Improving Human Experience in Health Care: Now More Than Ever, We Must Focus on People

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Perm J 2024;28:24.125 • <https://doi.org/10.7812/TPP/24.125>
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Following the seminal reports on health care quality and safety in the United States,^{1,2} there have been substantial efforts made toward developing the field of health care quality, development of the fields of implementation and complexity sciences,^{3,4} and efforts to improve health care quality and related outcomes in the United States and internationally. This has included the work of the Institute for Healthcare Improvement⁵ and patches of success realized by a number of organized improvement efforts (of which only a partial list are referenced here).⁶⁻¹² However, after nearly three decades of work, we have not realized substantive population-level improvements in the key areas of the Institute for Healthcare Improvement's Quadruple Aim,¹³ which encompasses outcomes (e.g. mortality, life expectancy, etc.), cost, patient experience, and workforce experience. Health care in the United States continues to become more expensive and less welcoming for the people engaging with it.¹⁴ Workforce engagement, burnout, and turnover are particularly concerning issues, with many health professionals leaving the workforce as health systems struggle to meet access demands of the populations they serve.¹⁵ Technological advances such as artificial intelligence (AI) may offer promising new possibilities, but also create new concerns and the need for ethical governance.¹⁶ The AI revolution also calls to memory many of the great promises made by past technological breakthroughs (such as the electronic health record) that have not yet been fully realized.¹⁷

Major implementation science frameworks,^{18,19} and hybrid improvement and implementation approaches²⁰ reinforce earlier thinking from clinical microsystems²¹ theory and more recent arguments from the developing sciences of learning health systems (LHS)²² and health care coproduction,²³ namely that a priority focus must be placed on the contexts in which the work is happening and the people interacting with it. Yet, a majority of our efforts, resources, and scholarship in the field has been focused on the mechanistic, procedural, process, and technological aspects of improvement, and comparatively less focused on the people involved.²⁴ Many quality and safety frameworks, value models, and measurement approaches²⁵⁻²⁸ include a focus on people by measuring patient experience, satisfaction, and engagement, but in practice, "people" and "experience" aspects have often come second to process improvement and cost containment. More recently, the Beryl Institute has defined the field of human experience²⁹ and developed a new value argument for prioritizing it in health care.³⁰ These efforts build upon the field of patient and family centered-care³¹ and align closely with developing the field of health care coproduction.³² The general thrust of these efforts calls to question the failure of our

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Thank you!

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